

CERVICAL SCREENING

A Pap smear (conventional cytology using glass slides/smear) is the common method of cervical screen. *If available*¹, use instead liquid-based cytology (LBC) and human papillomavirus (HPV) DNA testing. If cytology unavailable, use visual inspection with acetic acid (VIA).

Decide when the patient needs a cervical screen according to symptoms

Patient has no symptoms.

- If HIV negative: do 3 screens in a lifetime, each 10 years apart from age 30.
- If HIV positive: do cervical screen at HIV diagnosis (regardless of age), then 3-yearly.

Patient has symptoms.

- (Symptoms include: irregular or heavy vaginal bleeding, bleeding after sex or an abnormal vaginal discharge)
- Do cervical screen if symptoms are not responding to treatment, regardless of when routine screen was done.

Assess the patient needing a cervical screen

Assess	Note
Symptoms	• Manage symptoms as on symptom pages. If abnormal vaginal discharge ➤ 51; if abnormal vaginal bleeding ➤ 57. If routine cervical screen, delay until after treatment. • If abnormal vaginal discharge/bleeding not responding to treatment, do cervical screen at same visit.
Family planning	Assess patient's contraceptive needs ➤ 154. If pregnant, do cervical screen safely up to 20 weeks gestation.
Examination	• Do pelvic examination to check for pain on moving cervix and pelvic masses. If pain on moving cervix, treat for lower abdominal pain (LAP) syndrome ➤ 44. If mass, refer. • Do speculum examination to look for abnormalities of cervix: if any lesion/mass/polyp/erosion/ulcer/sore, avoid cervical screening and instead refer same week for colposcopy/biopsy.
HIV	Test for HIV ➤ 110. If HIV positive, give routine HIV care ➤ 111, and repeat cervical screening 3 yearly. If negative, consider need for PrEP ➤ 106.
Human papillomavirus (HPV) DNA test	<i>If liquid-based cytology (LBC) available</i> ¹ , also request HPV DNA test on same specimen.

Health for All

➤ 50

Advise the patient needing a cervical screen

- Educate that cervical cancer is a disease that affects the mouth of the womb. Certain types of HPV cause cervical cancer. HPV is transmitted sexually and can persist for years. Emphasise condoms.
- Cervical screening is able to prevent cervical cancer as it detects changes in the cervix years before cancer develops. Colposcopy is a closer examination of the cervix to confirm these abnormal changes.
- Advise that smoking increases the risk of cervical abnormalities. If patient smokes, encourage to stop ➤ 141.
- Advise patient to return if symptoms of cervical cancer (abnormal vaginal bleeding, vaginal discharge) occur.

Manage the patient according to results:

If specimen unsatisfactory or result not found, repeat cervical screen within 3 months.

Normal If available, check HPV DNA result:		Abnormal	
HPV DNA negative or not done	HPV DNA positive		
Cervical screen negative • Explain that patient has <i>no</i> abnormal changes of her cervix. • If HPV negative, explain that patient currently does not have the virus that can cause cancer changes. • If HIV negative: repeat after 10 years if < 3 previous routine screens. • If HIV positive: repeat screen after 3 years.	Cervical screen positive • If abnormal Pap smear/LBC/VIA, explain that patient has changes on her cervix that need further examination to check for cancer. • If normal Pap smear/LBC/VIA but HPV DNA positive, explain patient does not have cancer but needs referral as HPV can cause cancer. • If VIA is positive or HPV DNA positive for HPV types 16 and 18: refer for cryotherapy/LLETZ. • If abnormal Pap smear/LBC, VIA suspicious for cancer or HPV DNA positive for other HPV types: refer for colposcopy. • Repeat screen in 1-3 years according to colposcopy findings/management needed.		

¹These tests are only available in designated pilot facilities.